

Women's Specialists of Clear Lake

Dr. Geoffrey H. Klein, MD · FACOG · DABOM · 400 W Medical Center Blvd, Suite 300, Webster, TX 77598 · (281) 557-0300

External Cephalic Version (ECV)

A safe procedure to gently turn a breech baby head-down -- giving you a better chance at a vaginal birth

WHAT IS AN ECV?

- A procedure where your provider uses **firm hands on your belly** to gently turn the baby from breech (or sideways) to head-down
- Usually performed at **37 weeks of pregnancy** -- when baby is mature but still has room to turn
- Done in the **hospital**, with continuous fetal monitoring and an OR available in case an emergency cesarean is needed
- Takes only a **few minutes**, though setup and monitoring make it a half-day visit
- **Success rate: about 50-60%** overall (higher in women who have had a baby before)
- Successful ECV greatly **lowers your chance of needing a cesarean**

WHO IS A GOOD CANDIDATE?

- ✓ **Single baby** in breech, transverse, or oblique position at 36+ weeks
- ✓ **Adequate amniotic fluid** and a reactive non-stress test
- ✓ **No placenta previa** or other reason a vaginal birth would be unsafe
- ✓ **No prior classical (vertical) cesarean** or full-thickness uterine surgery
- ✓ **Intact membranes** (water hasn't broken)
- ✓ Even **after a low-transverse cesarean**, ECV can be considered with similar success rates
- ✓ **Better odds** if you have had a prior vaginal birth, a posterior placenta, plenty of fluid, or a complete (rather than frank) breech

WHEN ECV IS NOT RECOMMENDED

- **Placenta previa** or vasa previa
- **Prior classical or vertical uterine incision**
- **Active labor** with ruptured membranes or significantly engaged breech
- **Severe oligohydramnios** (very low fluid)
- **Non-reassuring fetal monitoring** or signs of distress
- **Significant fetal anomaly**, hyperextended head, or multiple gestation (twins) -- usually
- **Placental abruption**, severe preeclampsia, or HELLP syndrome
- **Fetal growth restriction** -- in most cases

RISKS TO KNOW ABOUT

- Overall **complication rate is about 6%** -- most are minor and resolve quickly
- **Temporary changes in fetal heart rate** (most common; typically resolve within minutes)
- **Vaginal bleeding** or rupture of membranes
- **Placental abruption** -- about 1 in 1,200 attempts
- **Emergency cesarean delivery** -- about 1 in 300 attempts
- **Stillbirth** -- about 1 in 5,000 attempts (very rare)
- **Small fetomaternal blood transfer** -- if you are Rh-negative, you will get **Rhogam** afterward
- Procedure can be **uncomfortable** but is usually tolerated without anesthesia

HOW TO PREPARE

- ✓ **Drink plenty of water** for 24 hours before -- may help by increasing amniotic fluid
- ✓ **Light breakfast** is OK; some hospitals ask you not to eat 2-8 hours before in case a cesarean is needed
- ✓ Wear **loose, comfortable clothing** -- you'll change into a gown
- ✓ Bring your **partner or support person** and your hospital paperwork
- ✓ Bring your **car seat and hospital bag** -- if labor begins or a cesarean is needed, you may not go home
- ✓ Plan to be at the hospital for **3-4 hours total**
- ✓ Avoid driving home alone if you receive medication that may cause sedation

WHAT TO EXPECT DURING & AFTER

- ✓ **Ultrasound** to confirm baby's position, fluid level, and placenta
- ✓ **Fetal heart-rate monitoring** before, during, and after
- ✓ **Terbutaline injection** (a medication to relax the uterus) is usually given 15-30 min beforehand
- ✓ Provider applies **steady, firm pressure** on your belly to roll baby into head-down position
- ✓ Procedure stops if you have **significant pain** or baby's heart rate changes
- ✓ Up to **2-4 attempts** may be tried; if unsuccessful, a repeat ECV can be offered another day
- ✓ **Monitoring continues 30-60 minutes** after; you go home if all is reassuring
- ✓ After successful ECV, **routine prenatal care resumes** -- we wait for spontaneous labor (no immediate induction)

⚠ **CALL OR GO TO LABOR & DELIVERY IMMEDIATELY FOR**

⚠ Vaginal bleeding more than light spotting

⚠ Severe abdominal pain

- ⚠ Leaking fluid (possible rupture of membranes)
- ⚠ Decreased or absent fetal movement
- ⚠ Regular contractions or low back pain
- ⚠ Lightheadedness, fainting, or rapid heartbeat
- ⚠ Fever or chills
- ⚠ Any concern that something is wrong -- trust your instincts

A Reasonable Step Toward a Vaginal Birth

ECV is one of the most evidence-based ways to avoid a cesarean delivery for a breech baby. About half to two-thirds of attempts succeed, and serious complications are rare. Even if it doesn't work, you still have safe options -- a planned cesarean, or in some cases, a vaginal breech delivery. Bring your questions and your birth preferences to your prenatal visit, and we will help you weigh the choice that's right for you and your baby.